Extracorporeal Shock Wave & RPW Referral / Treatment

PATIENT NAME: **DUMERVIL, PANDORA**

PATIENT SIGNATURE: 

DATE OF SERVICE: **10/27/2026** DATE OF ACCIDENT: **06/06/2026**

LOCATION: _____

PROCEDURE # _____1 _____2 _____3 _____4 _____5

____NECK __✓__LOWBACK _____ THORACIC __LT/RTSHOULDER ___LT/RTKNEE

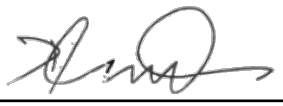
____ OTHER_____

SHOCKWAVE THERAPY

DX CODES:

- ☐ M54.2 Cervicalgia
- ☐ M54.6 Pain in Thoracic
- ☑ M54.S Low Back Pain
- ☐ M54.41 Lumbago with Sciatica R side
- ☐ M54.42 Lumbago with Sciatica L side
- ☐ M25.561 Pain in Right Knee Joint
- ☐ M25.562 Pain in Left knee joint
- ☐ M25.511 Pain in Right Shoulder
- ☐ M25.512 Pain in Left shoulder
- ☐ M79.604 Pain in Right Leg
- ☐ M79.605 Pat in Left Leg

OTHER_____

Referring Physician: Dr. David Carmili 

Name **Signature**

Consent for Shock Wave Therapy Treatment (ESWT) and Hold Harmless Agreement

I understand that I am a patient of _____

Cooperation with treatment:
In order, for Shock Wave Therapy treatment to be effective, I must come to scheduled appointments unless there are unforeseen circumstances. I understand and agree to cooperate with and perform the home physical therapy program intended for me. If I have trouble with any part of my treatment program, I will discuss it with my provider.

No warranty:
I understand that there are no guarantees regarding a cure for or improvement in my condition. understand that my provider will outline and discuss the goals of Shock Wave Therapy treatment for my condition and will discuss treatment options with me before I consent to treatment.

Informed consent for treatment:
The term "informed consent" means that the potential risks, benefits have been explained to me.

Potential risks:
Side effects from ESWT are limited to mild bruising, swelling, pain, numbness or tingling in the treated area, and the recovery is minimal compared with that of surgical intervention. This discomfort is usually temporary; if it does not subside in a reasonable time, I agree to contact my physician.

Potential benefits:
I may experience an improvement in my symptoms and an increase in my ability to perform daily activities. I may experience increased strength, awareness, flexibility, and endurance in my movements. I may experience decreased pain and discomfort. I should gain a greater knowledge about managing my condition and the resources available to me

Alternatives: If I do not wish to participate in the ESWT program, I will discuss my medical, surgical, or pharmacological alternatives with my physician or primary care provider.

Payment: I understand that I am responsible for any charges not covered by insurance.

I have read the above information and consent to ESWT.

I hereby release and hold harmless _____ for any loss/judgement and/or harm which might come to me because of this Shock Wave Therapy treatment (ESWT). I waive all claims associated with this treatment which I may have against _____, his employees, officers, directors, and agents.

Patient Name (Print): **DUMERVIL, PANDORA**

Patient Signature: 

Parent/Guardian Signature: _____

Date: **10/27/2026**

Patient Name: **DUMERVIL, PANDORA**

MR#: _____ Date of Accident: **06/06/2026**

MEDICAL LEIN

Attorney Name:

_____

_____

Patient Name: **DUMERVIL, PANDORA**

I hereby authorize and direct my attorney. to pay directly to _____ such sums as may be due and owing for professional services rendered to me both by reason of this accident and by reason of any other bills that are due to the provider and to withhold such sums from any settlement of judgment as is necessary to adequately protect the provider.

I hereby further give a lien to the provider on any proceeds to which I may become entitled as a result of any settlement of judgment in any claim or litigation arising our of the injuries for which I have been treated of injuries in connection therewith, whether such proceeds are remitted directly to me or to you, mv attorney.

I fully understand that I am directly responsible to the provider for all professional bills submitted by the provider for services rendered to me by the provider and that this agreement is made solely for the providers' additional protection and in consideration of the provider awaiting payment. I further understand that such payment is not contingent on any settlement, judgment, or verdict by which I may eventually recover said fee

Attorney agrees to notify the doctors immediately of the name and contacting information of any attorney substituted in his or her place.

DUMERVIL, PANDORA	**10/27/2026**
Print Patient Name	Date
Patient's Signature	Signature of Parent/ Guardian

ACKNOWLEDGEMENT OF ASSIGNMENT & LIEN BY ATTORNEY

The undersigned being the attorney of record on his own behalf and on behalf of any other attorney or attorneys who are associated with the undersigned or who are substituted in his stead for the above patient, does hereby agree to observe all the terms of the above and agrees to withhold such sums from any settlement, judgment or verdict as may be necessary to adequately protect,

_____

	10/27/2026
Attorney 's Signature	Date

* NOTE TO ATTORNEY

Please sign and return one copy to the provider's office; keep a copy for your records

NEW YORK MOTOR VEHICLE NO-FAULT INSURANCE LAW
ASSIGNMENT OF BENEFITS FORM

(FOR ACCIDENTS OCCURRING ON AND AFTER
3/1/02)

I, **DUMERVIL, PANDORA** ("Assignor") hereby assign to **NORMAN MEDICAL, P.C.** ("Assignee")
(Print patient's name) (Print hospital or health care provider name)

all rights privileges and remedies to payment for health care services provided by assignee to which I am entitled under Article 51 (the No-Fault statute) of the Insurance Law.

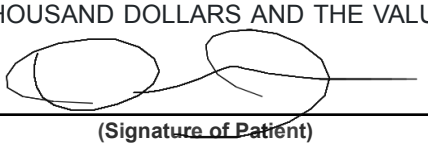
The Assignee hereby certifies that they have not received any payment from or on behalf of the Assignor and shall not pursue payment directly from the Assignor for services provided by said Assignee for injuries sustained due to the motor vehicle accident which occurred on **06/06/2026** , not withstanding any other agreement
(Print accident date)

to the contrary.

This agreement may be revoked by the assignee when benefits are not payable based upon the assignor's lack of coverage and/or violation of a policy condition due to the actions or conduct of the assignor.

ANY PERSON WHO KNOWINGLY AND WITH INTENT TO DEFRAUD ANY INSURANCE COMPANY OR OTHER PERSON FILES AN APPLICATION FOR COMMERCIAL INSURANCE OR A STATEMENT OF CLAIM FOR ANY COMMERCIAL OR PERSONAL INSURANCE BENEFITS CONTAINING ANY MATERIALLY FALSE INFORMATION, OR CONCEALS FOR THE PURPOSE OF MISLEADING, INFORMATION CONCERNING ANY FACT MATERIAL THERETO, AND ANY PERSON WHO, IN CONNECTION WITH SUCH APPLICATION OR CLAIM, KNOWINGLY MAKES OR KNOWINGLY ASSISTS, ABETS, SOLICITS OR CONSPIRES WITH ANOTHER TO MAKE A FALSE REPORT OF THE THEFT, DESTRUCTION, DAMAGE OR CONVERSION OF ANY MOTOR VEHICLE TO A LAW ENFORCEMENT AGENCY, THE DEPARTMENT OF MOTOR VEHICLES OR AN INSURANCE COMPANY, COMMITS A FRAUDULENT INSURANCE ACT, WHICH IS A CRIME, AND SHALL ALSO BE SUBJECT TO A CIVIL PENALTY NOT TO EXCEED FIVE THOUSAND DOLLARS AND THE VALUE OF THE SUBJECT MOTOR VEHICLE OR STATED CLAIM FOR EACH VIOLATION.

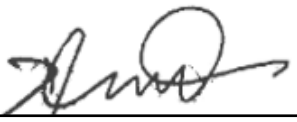
DUMERVIL, PANDORA
(Print name of Patient)


(Signature of Patient)

10/27/2026
(Date of signature)

(Address of Patient)

NORMAN MEDICAL, P.C.
(Print name of Provider)


(Signature of Provider)

714 SENECA AVENUE

RIDGEWOOD, NY 11385
(Address of Provider)

10/27/2026
(Date of signature)

New York Rehab Care PT PC

2220 Flatbush Ave
Brooklyn, NY 11234

Phone: (718) 313-4300
Fax: (718) 313-4261
Email: nyrehabcare@gmail.com

Patient Summary

CASE #1070

Last Name:	DUMERVIL	First Name:	PANDORA	Middle Name:	M

SSN:	105-98-4368	Address:	261 LENOX RD	Primary Phone:	(646) 289-1355
Date Of Birth:	05/02/1994	Apt:	2C	Secondary Phone:	---
Age:	32 Years Old	City:	BROOKLYN	Email:	---
Sex:	Female	State:	NY		
Marital Status:	Single	Zip:	11226		

POLICY HOLDER

First Name:	PANDORA	Date Of Birth:	05/02/1994	Primary Phone:	(646) 289-1355
Last Name:	DUMERVIL	Age:	32 Years Old	Secondary Phone:	---
		Sex:	Female	Email:	---

INSURANCE (PRIMARY)

Ins Name:	Integon National Insurance Company	Case Type:	No Fault	Phone:	(800) 848-1777
Address:	P.O BOX 2989	Role:	Driver	Fax:	---
City:	Clinton	D.O.A:	06/06/2026	Email:	---
State:	IA	Policy #:	65156930		
Zip:	52733	Claim #:	260623831		

PERSONAL INJURY FIRM INFORMATION

Firm:	Robert J. Renna, P.C.	Phone:	(718) 422-1003
Address:	50 Court St	Fax:	---
City:	Brooklyn	Email:	robert.renna@r-ennalaw.com
State:	NY		
Zip:	11201		

PATIENT: DUMERVIL, PANDORA	**DOB:** 05/02/1994	**SEX:** Female
DOCTOR: MATIAS NATARA, MONIQUE	**VISIT:** 09/23/2026	**DOA:** 06/06/2026

Initial Visit

Accident type: MVA

Pandora Dumervil is a 32 year old female born on 05/02/1994 . In general, the patient considers herself to be in good health.
Pandora Dumervil was involved in an automobile collision on Saturday, June 6, 2026 involving 2 vehicle(s). The collision occurred in Brooklyn (on Vanderbilt). The patient stated that she was struck from the left side. The patient informed me that at the time of the collision she was a driver. Pandora also informed me that she did not loose consciousness during the accident. During the collision, the patient's head hit the side window.
Other: Chest hit Steering Wheel Furthermore, Pandora states that she was wearing a shoulder strap seat belt.
After the accident, Pandora went to CUNY DownState- walked to hospital Hospital, where she was treated and released the same day. The patient had X-Rays taken over her right knee and lower back.

Complaints

Pain in Head: 7 out of 10. Aggravated by Physical Activity (ADLs). Alleviated by Advil. Type of pain: Sharp,Aching.
Pain in Lower back: 5 out of 10. Aggravated by Bending back,Bending down,General Movement,Getting up from seating position,Going down the stairs,Going up the stairs,Lumbar rotation,Physical Activity (ADLs). Alleviated by Clinical Treatment,Laying Down. Type of pain: Sharp,Aching,Stiffness.
Pain in Right knee: 5 out of 10. Aggravated by Bending down,General Movement,Going down the stairs. Alleviated by Clinical Treatment. Type of pain: Cramping,Aching.
Other complaints: Headaches.

History

Medical and Mental Health History

Allergic to None or nka.
Developmental Concerns: None reported
Medical History: NONE
Surgical History: Right Knee August 27, 2026
Hospitalization History: NONE
Psychiatric/Psychological History: None reported
Diagnostic Studies/Medical Records: none
Previous Accidents: Patient denied having any previous accidents

Current Medication

Non-Psych Medications: n/a

Social History

Substances: Patient denied use of any substances
Cannabis: No.
Alcohol: None.
Tobacco: None
Caffeine: No.

Legal History: Patient denied any significant legal history
Marital Status: Single.
Additional information: Patient presents with resiliency and the capacity to adjust and adapt to the medical and psychological effects associated with the reported accident. Patient continues to experience symptoms that may contribute to functional limitations and interfere with activities of daily living. Patient remains engaged in the treatment plan and continues to participate in physical therapy, rehabilitation care, and supportive services as clinically indicated. Continued treatment is considered medically appropriate based on the patient's current clinical presentation, reported symptoms, functional status, and response to treatment. The patient's condition, functional limitations, and progress will continue to be monitored, and the treatment plan will be modified as clinically appropriate. Patient was encouraged to remain compliant with all recommended treatment and follow-up care. Prognosis and disability status will be reassessed based upon clinical progress and response to treatment. Continued supportive and rehabilitative care remains appropriate at this time.

Review of Systems

Positive for Const. (Health in General): Negative

Positive for Ears, Nose, Mouth & Throat: Negative

Positive for C-V (Heart & Blood Vessels): Negative

Positive for Resp.(Lungs & Breathing): Negative

Positive for GI (Stomach & Intestines): Negative

Positive for GU (Kindney & Bladder): Negative

Positive for MS (Muscles, Bones, Joints, Hip, Neck, Knee, Elbow, Ankle, Hand): Joint pain, Aching muscles, Back pain

Positive for Neurologic (Brain & Nerves): Negative

Positive for Const. (Health in General): Negative

Positive for Allergic/Immunologic: Negative

Physical Exam

General

Height: 5 FT | 2 inch
Weight: 106 LB
Dominant Hand: Right

General appearance: Well developed, well nourished, alert and cooperative, and appears to be in no acute distress.
Psychiatric: Oriented to person, place, and time.
Heent: Normocephalic, atraumatic, ENT grossly normal
Head: Normocephalic.
Eyes: PERRL, EOMI. Fundi normal, vision is grossly intact.
Ent: External auditory canals and tympanic membranes clear, no nasal discharge, oral cavity and pharynx no inflammation, swelling, exudate, or lesions.
Neck: Neck supple, non-tender WITH PALPATION, NEGATIVE lymphadenopathy, masses or thyromegaly.
Chest: No deformities, no pain on palpation.
Lungs: Clear to auscultation and percussion without rales, rhonchi, wheezing or diminished breath sounds.
Heart: wnl
Cardiac: Normal S1 and S2. No S3, S4 or murmurs. Rhythm is regular. No carotid bruits.
Abdomen: Positive bowel sounds. Soft, nondistended, nontender. No guarding or rebound. No masses. + on bowel sounds.

Extremities: No peripheral edema, cyanosis or pallor. Extremities are warm and well perfused. Capillary refill is less than 2 seconds.
Skin normal color, texture and turgor with no lesions or eruptions.
Neurological: CN II-XII intact. Strength and sensation symmetric and intact throughout. Reflexes 2+ throughout. Cerebellar testing normal.
Kyphoscoliosis: -
Lumbar Lordosis: Normal
Muscle Spasm: +
Cardiovascular: Denies chest pain, night awakening due to trouble breathings, palpitations.
Endocrine: Denies cold intolerance, appetite changes, hair changes.
Gastrointestinal: Denies abdominal pain, change in bowel habits, constipation, nausea, vomiting, rectal bleeding.
Genitourinary: Denies blood in urine, painful urination, loss of bladder control, urinary retention.
Hematology: Denies easy bruising, enlarged lymph nodes, prolonged bleeding.
Musculoskeletal: Denies muscle wasting. See HPI.

Radiology Results

Image Studies. The Patient presented with results of his imaging studies which showed:

Lumbar

Other: LEFT KNEE 1. JOINT EFFUSION 2. Grade I-II LCL sprain. 3. Heterogeneous, thickened and partially torn ACL. 4. Iliotibial band syndrome with hypertrophy of gerdy's tubercle of the proximal fibula. 5. Narrowing of the femorotibial compartments. 6. Tearing posterior horn of the medial meniscus. Discoid shape lateral meniscus. 7. Quadriceps and patellar tendinosis/tendonitis. Supra and infrapatellar plica. Synovitis of Hoffa's infrapatellar fat pad. LUMBAR 1. Straightened lumbar lordosis. 2. Deep thoracolumbar fascitis. 3. Fibroid type changes of the uterus with a thickened junctional zone. PAP smear of the cervix and pelvic sonography is recommended. 4. L4-L5 dise demonstrates a dise bulge with a central 2-3 mm disc herniation with thecal sac flattening. There are multi-level hypertrophie facet change, ligamentum flavum hypertrophy with lateral recess and neural foraminal narrowing. The visualized nerve roots reveal no evidence of structural abnormality. 5. L5-S1 disc demonstrates a disc bulge with a central 2-3 mm disc herniation with thecal sac flattening. There are multi-level hypertrophic facet change, ligamentum flavum hypertrophy with lateral recess and neural foraminal narrowing. The visualized nerve roots reveal no evidence of structural abnormality. 6. LI-L2 through L3-L4 discs are normal in contour without bulging, herniation or neural impingement.

Extremities MR

Incidental Findings: MRI OF RIGHT KNEE WITHOUT CONTRAST IMPRESSION: 1. Grade I sprain of the anterior cruciate ligament. 2. Grade II meniscal injury involving the body and posterior horn of the medial and lateral menisci, as described above 3. Strip of knee joint effusion with suprapatellar extension and synovial thickening. MRI OF LUMBAR SPINE WITHOUT CONTRAST IMPRESSION: 1. L3-L4: Diffuse disc herniation with bilateral foraminal disc protrusions is seen. Bilateral neural foramina are stenosed. Spinal canal is normal. Foraminal disc protrusions measures about 3.9mm. 2. L4-L5: Diffuse disc herniation with bilateral foraminal disc protrusions is seen. Bilateral neural foramina are stenosed. Spinal canal is normal. Foraminal disc protrusions measures about 3.4mm. 3. L5-S1: Diffuse disc herniation of 2.2mm is seen. Ventral thecal sac is effaced. Bilateral neural foramina and spinal canal are normal MRI CERVICAL SPINE WITHOUT CONTRAST IMPRESSION: 1. C2-C3: Left foraminal disc protrusion is seen. Left neural foramen is effaced. Spinal canal is normal. Left foraminal disc protrusion measures 1.9mm. 2. C3-C4: Right foraminal disc protrusion is seen. Right neural foramen is effaced. Spinal canal is normal. Right foraminal disc protrusion measures 1.7mm. 3. C5-C6: Diffuse disc herniation of 1.3mm is seen. Bilateral neural foramina and spinal canal are normal. 4. C6-C7: Central disc protrusion of 1.4mm is seen. Anterior thecal sac in indented. Bilateral neural foramina, spinal canal is normal.

Range of Motion

PATIENT: DUMERVIL, PANDORA | **DOB:** 05/02/1994 | **SEX:** Female

DOCTOR: MATIAS NATARA, MONIQUE | **VISIT:** 09/23/2026 | **DOA:** 06/06/2026

Cervical Spine				
	Degree	% of Normal	Normal	Condition
Flexion	45	90%	50	Without any pain
Extension	55	92%	60	Without any pain
Lateral Left Bending	40	89%	45	Without any pain
Lateral Right Bending	40	89%	45	Without any pain
Lateral Left Flexion	45	100%	45	Without any pain
Lateral Right Flexion	40	89%	45	Without any pain
Left Rotation	75	94%	80	Without any pain
Right Rotation	75	94%	80	Without any pain

Lumbosacral Spine				
	Degree	% of Normal	Normal	Condition
Flexion	80	89%	90	With pain
Extension	20	80%	25	With pain
Lateral Left Rotation	30	67%	45	With pain
Lateral Right Rotation	30	67%	45	With pain
Lateral Left Bending	25	83%	30	With pain
Lateral Right Bending	20	67%	30	With pain

Right Knee				
	Degree	% of Normal	Normal	Condition
Flexion	125	93%	135	With pain

Ortho Strength Exam

Knees

	Left	Right
Flexion	/5	5 /5
Extension	/5	5 /5
Knees other	/5	/5

Orthopedic Tests

Ankles

Special test	
Talar Tilt	All wnl
Anterior Drawer	All wnl
Thompson	All wnl
Point Tenderness	All wnl

Knees

Crepitus	Positive right
Point Tenderness Knees	Right medial joint line Right lateral joint line Right suprapatellar

Hips

Special test / Maneuvers	
Trendelenburg Sign	All wnl
Thomas Test	All wnl
Ober's Test	All wnl
Romberg's Test	All wnl
Provocative SI Test	
Faber (Patrik's)	All wnl
Gaelsen	All wnl
Hip Thrust	All wnl
Distraction	All wnl
Compression	All wnl
Gait Patterns	
Trendelenburg	All wnl
Broad Based (ataxia)	All wnl
Shuffling	All wnl
High Stepping (drop foot)	All wnl
Antalgic (reduce stance phase)	All wnl
InToeing (femoral anteversion)	All wnl

Quadriceps Avoidance	All wnl

Shoulders

Shoulder	All wnl
Spurling	All wnl
Hawkins Test	All wnl
Neer's Test	All wnl
Empty Can Test	All wnl
Speeds Test	All wnl
Yergason's Test	All wnl
Gerber's Lift-off Test	All wnl
Belly Press Test	All wnl
Scapular Winging	All wnl
Scapulothoracic Compensation	All wnl
Point Tenderness	All wnl

Elbows

Pain with resisted wrist extension	All wnl
Pain with resisted wrist flexion	All wnl
Varus Stress	All wnl
Valgus Stress	All wnl
Effusion	All wnl
Point Tenderness	All wnl

Wrists

Tinel's Sign	All wnl
Phalen's Test	All wnl
Finkelstein's Test	All wnl
Thenar Atrophy	All wnl
Weak Pinch Strength	All wnl
Weak Grip Strength	All wnl
Effusion	All wnl
Point Tenderness	All wnl

Cervical Spine

Spurling's Test	All wnl
Gait Instability	All wnl
Loss of hand dexterity	All wnl
Change Bowel / Bladder	All wnl
Point Tenderness	Bilaterally midline spine Bilaterally paraspinal muscles Bilaterally mastoid process Bilaterally occiput Bilaterally upper trapezium

Lumbar

Straight Leg Raise Test	Positive bilaterally
Point Tenderness	Bilaterally midline spine Bilaterally paraspinal muscles Bilaterally sciatic notch Bilaterally iliac crest Bilaterally greater trochanter

Cervical Muscles

	Symmetrical No

Thoracic Spine	Negative muscle spasm of paraspinal muscles Negative tenderness upon palpation Negative limitation of rom

Neurological

Deep tendon reflexes

	Left	Right
Cervical		
Biceps	2 /5	2 /5
Brachioradialis	2 /5	2 /5
Triceps	2 /5	2 /5
Lumbar		
Patellar	2 /5	2 /5
Achilles	2 /5	2 /5

Treatment Plan

Physical therapy - 2-3 per week for 4-6 weeks. Evaluate and treat, Rom exercises.

Imaging:

MRI of the completed are recommended based and patient complains and objective findings.

Re-evaluation in 4 week .

RX

Medications

Lidocaine: Ointment 5%

Naprosyn: 375/20 mg 30 days

Other medications: CYCLOBENZAPRINE 15MG 30 TABS HS

Supply Prescribed:

Orthopedic Car Seat.
WEARABLE PEMF DEVICE POWER PRESSURED REDUCING MATTRESS R KNEE CUSTOM

Patient Education:

Proper body mechanics to promote healing and avoid further injuries, Importance of attending physical therapy sessions , follow-up consultations and diagnostic studies, Patient was reassured to return sooner than scheduled appointments if pain changes or worsen.

Diagnosis

G89 Pain,

NM WELLNESS IN FAMILY HEALTH NP PLLC (2220 Flatbush Ave, Brooklyn NY, 11234 (11))

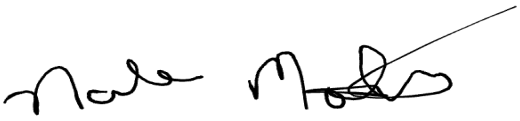
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DOCTOR: MATIAS NATARA, MONIQUE	**VISIT:** 09/23/2026	**DOA:** 06/06/2026

M25.561 Pain in right knee,
M54.16 Lumbar Radiculopathy,
R51.9 Headache, unspecified,
M54.50 Low back pain, unspecified.

Superbill

99204.

Np in family health Signature:



Name: MONIQUE MATIAS NATARA
Title: NP IN FAMILY HEALTH
License: # F342236